

Better care. Closer to home.

Nurse-led planned home care, with clinician review and consented family updates.

R5.5 million

Proposed 18-month pilot round

A three-platform software foundation. A focused care-delivery pilot to prove safety, repeat use and sustainable economics.

Gilbert Makinana · Akhanya IT Innovations · October 2026

Why I am building MyThuso

I was born and raised in Gugulethu.
My mother still lives there.
I work in Johannesburg.

Her monthly clinic trip means a taxi, a queue that lasts most of the day and a long walk home.

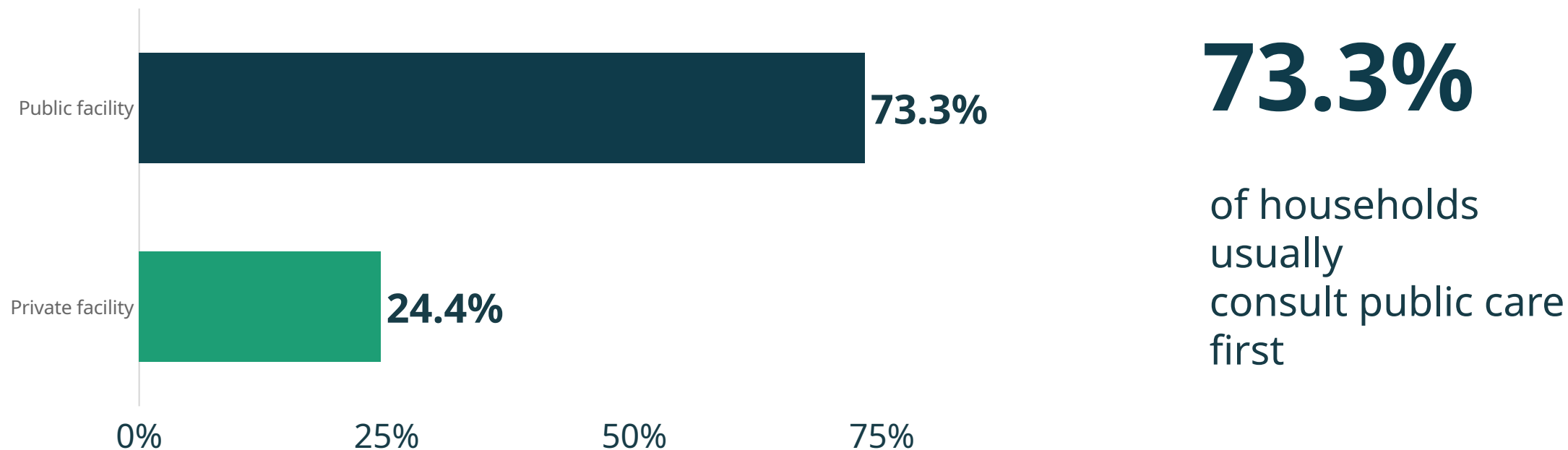
I lost my father during COVID.

“The first MyThuso for Mom customer will be my mother.”

Care close to a parent's home.
A trusted update for the family.

Most households turn to public care first

Usual first place of consultation, share of households, 2025

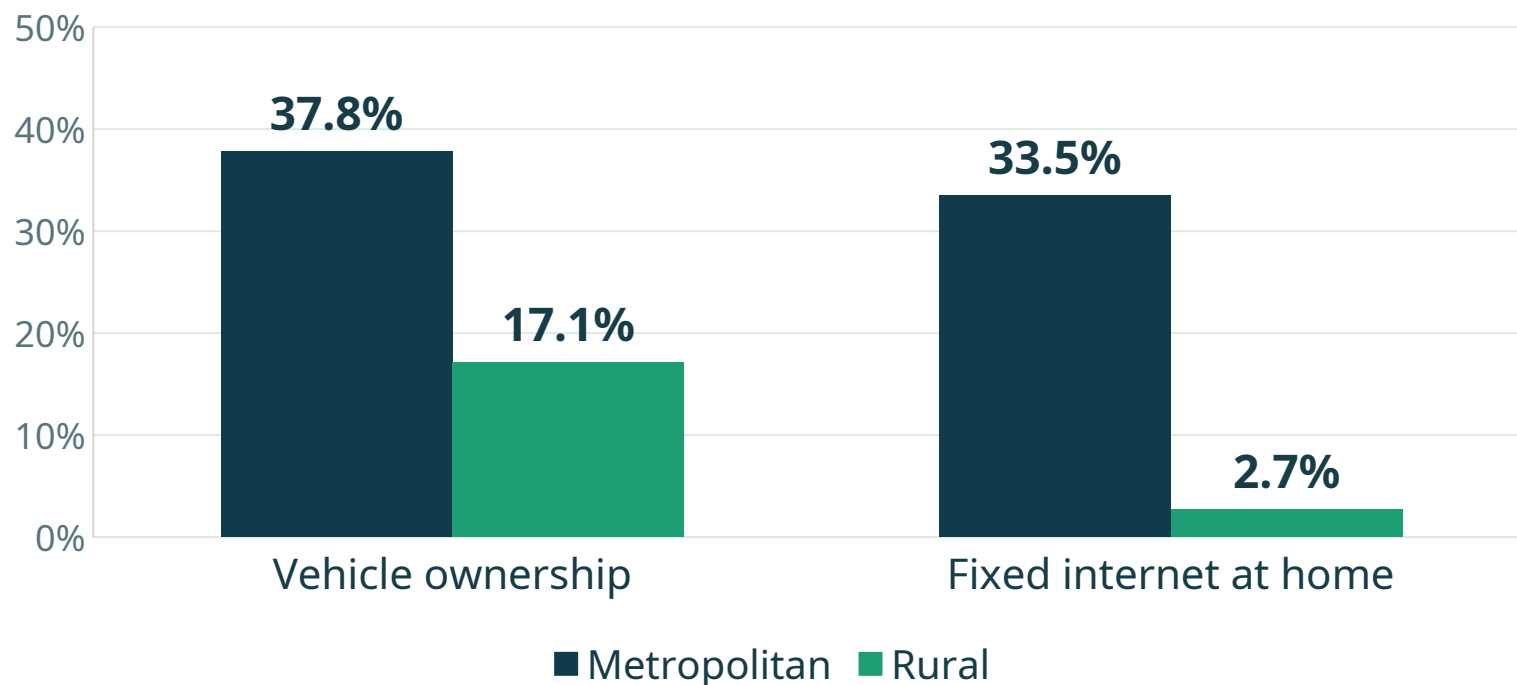


MyThuso aims to bring suitable planned care into the home, with clinical referral when needed.

Source: [Stats SA](#) GHS 2025, Figure 5.2, p20. Household first-contact preference. Other responses are not shown.

Rural care needs a local delivery model

Household access indicators, 2025



76.4%

of rural households
report mobile
internet access

Pilot priorities: assisted
booking and planned
local nurse routes.

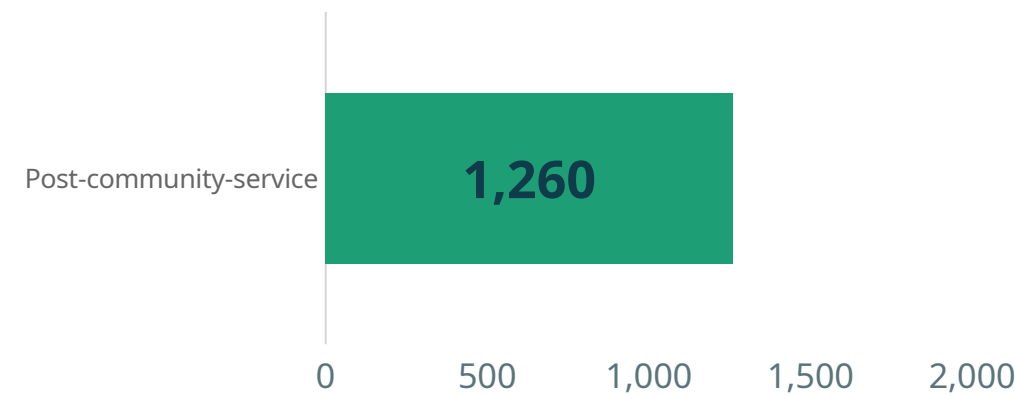
Source: [Stats SA](#) GHS 2025, Tables 14.3 and 19.1, pp56 and 67–68. Household indicators, not a clinical travel-time measure.

Trained clinicians can still be without work

Published unemployment evidence. The two snapshots use different dates and methods.

Doctors

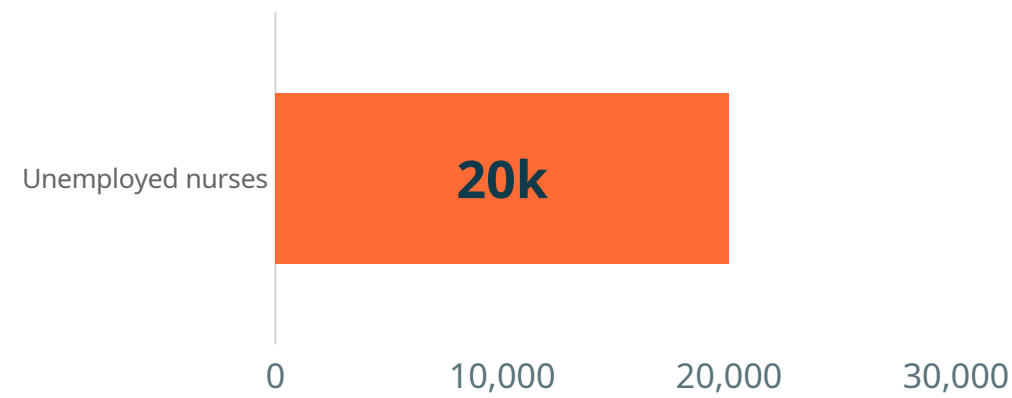
SAMATU survey, January 2026



Self-reported survey respondents, not a national unemployment total

Nurses

DENOSA estimate, May 2023



Historical union estimate, cited again in April 2025. No current official national total verified.

Sources: SAMATU statement, 20 Jan 2026 (COSATU archive); DENOSA estimate via News24/COSATU, May 2023; eNCA, 12 Apr 2025.

MyThuso connects the family to a care team

The proposed service turns a booking into a visit, a record and a clear next step

01

Book

A family requests a planned visit. The patient gives consent and chooses who receives updates.

02

Visit

A verified nurse attends within a defined service area. Each visit creates a paid care opportunity.

03

Review

A registered clinician reviews where clinically needed, with a clear urgent escalation route.

04

Follow through

A Health Passport records care. The patient and authorised family receive agreed updates and next steps.

Pilot design. Availability, response times, scope of practice and clinical escalation protocols require validation before launch.

The software exists. The service must be validated.

Product readiness based on the current repository; built does not mean clinically activated.

Built foundation

React web, native iOS and Android.

Role workspaces, consent and record workflows, clinician tools and simulated triage displays.

Recent improvement

Animated heart and oxygen/lung visuals for patients, nurses and doctors.

Web consultation layout places readings, patient details and medication requests beside a video placeholder.

Before patient launch

Identity/SMS and real sign-in; live booking, payment and dispatch; video and device connections.

Clinical staffing, secure real-data release and operating agreements.

Home care is an established market

MyThuso's proposed focus is family-funded continuity of care in a tightly defined launch area

Existing option	What the provider describes	Implication for MyThuso
Quoro Medical	Hospital-at-home with monitoring, virtual check-ins and in-person care	Integrated home care is already offered
Medwell	Home nursing, carers and recovery or frail-care services	Trust, staffing and service quality matter
Hello Doctor	Health advice and virtual clinical consultations	Remote access alone is not differentiation
MyThuso (proposed)	Family booking, verified visits, patient records and authorised family updates	Test continuity, transparent cost and local reliability

Sources: provider websites, checked 30 Sep 2026. Service descriptions are not independent evidence of outcomes or like-for-like prices.

A local pilot with a path to remote communities

Northern Johannesburg and Soweto first, as described on the website

First: reliable local delivery

Verify clinicians and cover a defined service area.

Measure completed visits, repeat use and contribution per episode.

Then: a rural service model

Test scheduled routes and assisted booking with a local partner.

Validate travel costs, connectivity and urgent referral before expansion.

The opportunity is closer care and sustainable clinical work. Each new area needs a model that can deliver both.

Launch geography: MyThuso website. Rural operating approach is a proposed next-stage pilot, not an existing service commitment.

R299 is a pricing test, not a margin guarantee.

Illustrative wound-care episode. Catalogued price and nurse share; other costs are revised cash assumptions.

Per completed visit	ZAR
Patient payment	299.00
Nurse payout	(224.00)
Card fee including assumed 15% VAT	(13.30)
Consumables / travel / refund reserve	(42.00)
Residual before doctor review and fixed costs	19.70
Doctor review when required (assumed)	(45.00)
Contribution if every visit is reviewed	(25.30)

R5.5m to prove the care model before hardware.

Revised 18-month funding envelope; all allocations are estimates, not committed supplier costs.

Use of funds	R million
Clinical leadership and medical oversight	1.20
Technology delivery and security engineering	1.35
Operations and patient support	0.72
Cloud, communications and video allowance	0.24
Standard equipment and calibration	0.20
Pilot variable-cost envelope	0.30
Legal, privacy, insurance and assurance	0.35
Patient acquisition and community partnerships	0.30
Finance and administration	0.14
Contingency and working-capital reserve	0.70

Three gates. An 18-month validation plan.

Proposed releases total R5.5m. Timing depends on clinical and real-data readiness.

R1.8m · Months 1–5

PREPARE

Appoint accountable clinical leads; secure insurance and protocols; complete identity, payments and privacy gates.

Gate: independent readiness review before enrolling patients.

R2.0m · Months 6–12

DELIVER

One tightly defined service area; 10–20 part-time nurses and 2–3 reviewing doctors, subject to agreements.

Gate: 500 completed visits with reviewed incidents and reconciled episode costs.

R1.7m · Months 13–18

VALIDATE

Target 3,000 cumulative visits; test repeat family care and retention.

Gate: sustainable contribution, clinical audit and evidenced route capacity before expansion.

Clinical and data safeguards before scale

Four proposed release gates for a responsible pilot

- | | | |
|-----------|--------------------------------|--|
| 01 | Clinical accountability | Named clinical leads, verified registrations, agreed scope and tested escalation |
| 02 | Bounded GilbertOne | Navigation and approved information first. Clinicians own clinical decisions. |
| 03 | Patient control | Consent, caregiver permissions, minimum necessary access and tested security |
| 04 | Validated expansion | Device intended use, evidence, applicable regulatory route and controlled AI updates |

Basis: HPCSA telehealth/AI guidance, POPIA and SAHPRA device guidance. These are proposed controls, not a compliance certification.

The team required to deliver

A software foundation with clinical leadership as a launch condition

Gilbert Makinana

Founder and software entrepreneur

Director, Akhanya IT Innovations

Akhanya development team supports the platform build.

Clinical appointments, committed provider capacity and partner arrangements remain evidence to secure before patient enrolment.

Clinical leadership

Clinical Governance Lead
Protocols, clinician vetting and incident review

Medical Director
Clinical oversight, escalation and AI governance

Impact measured from the first visit

The pilot should establish service quality, sustainable work and continuity of care

Question	Measure	How to interpret it
Can people access the service?	Completed visits and time to clinician	Report by service area and patient group
Does care continue safely?	Follow-up completion and escalation closure	Use clinical review, not app activity alone
Do families return?	Repeat use and plan retention	Show cohorts and reasons for cancellation
Can clinicians earn sustainably?	Net provider earnings and utilisation	Include travel, time and operating costs
Does the model sustain itself?	Cost and contribution per episode	Reconcile all costs and subscription visits

Proposed pilot measurement framework. Earlier diagnosis, fewer admissions and job creation remain outcomes to evaluate, not achieved results.

R5.5m to build a service families can trust.

Proposed seed partnership · a smaller, gated pilot before a separate scale-up round.

Fund clinical readiness. Prove reliable home care.

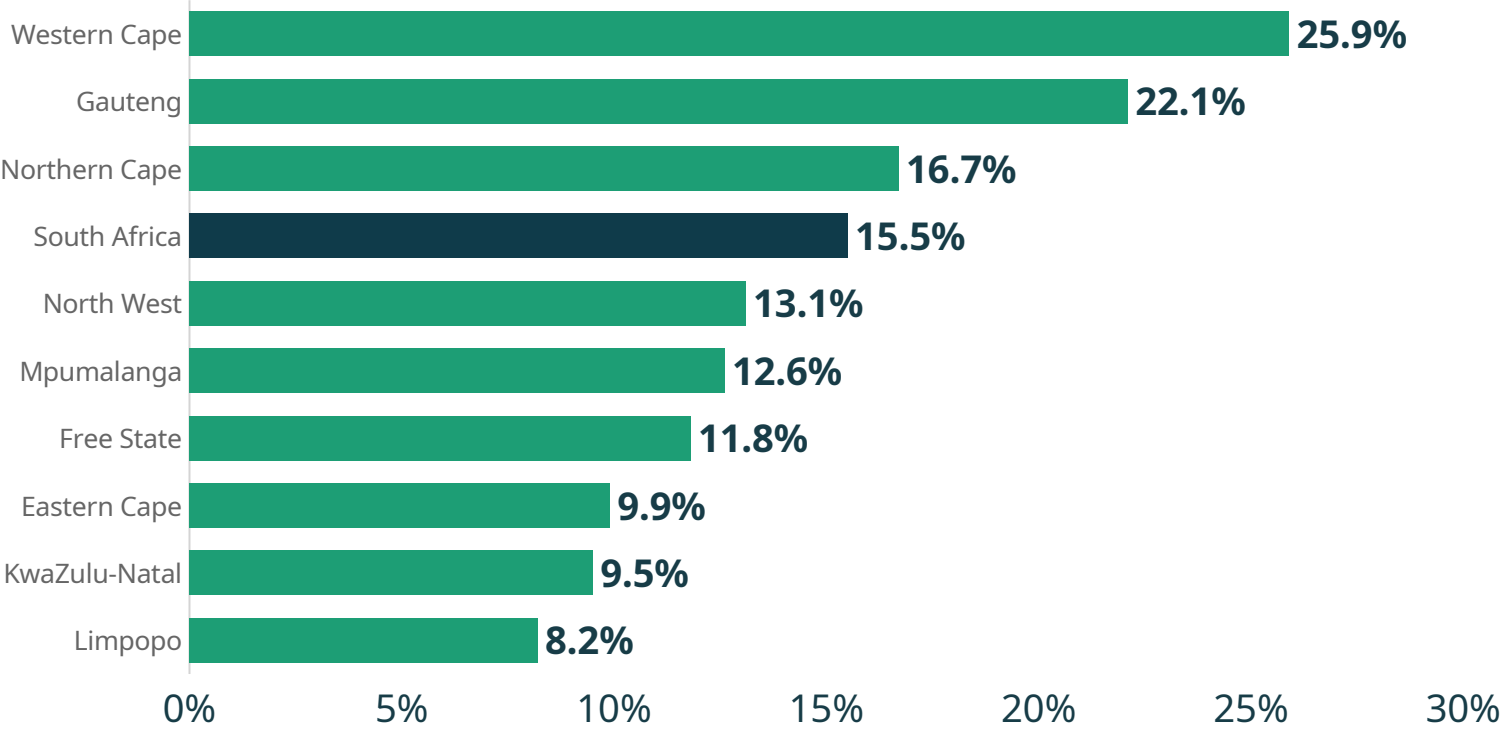
The existing software reduces the need to start again. Funding turns the foundation into an accountable operating service: clinicians, secure systems, local delivery and measured outcomes.

Custom Pod, Band, Home and Lab development is deferred. Geographic expansion and additional clinical AI require their own evidence and funding decisions.

Gilbert Makinana · gilbert@akhanya.co.za · +27 79 077 1591

Medical-aid coverage is limited and uneven

Share of individuals reporting medical-aid coverage, 2025



15.5%

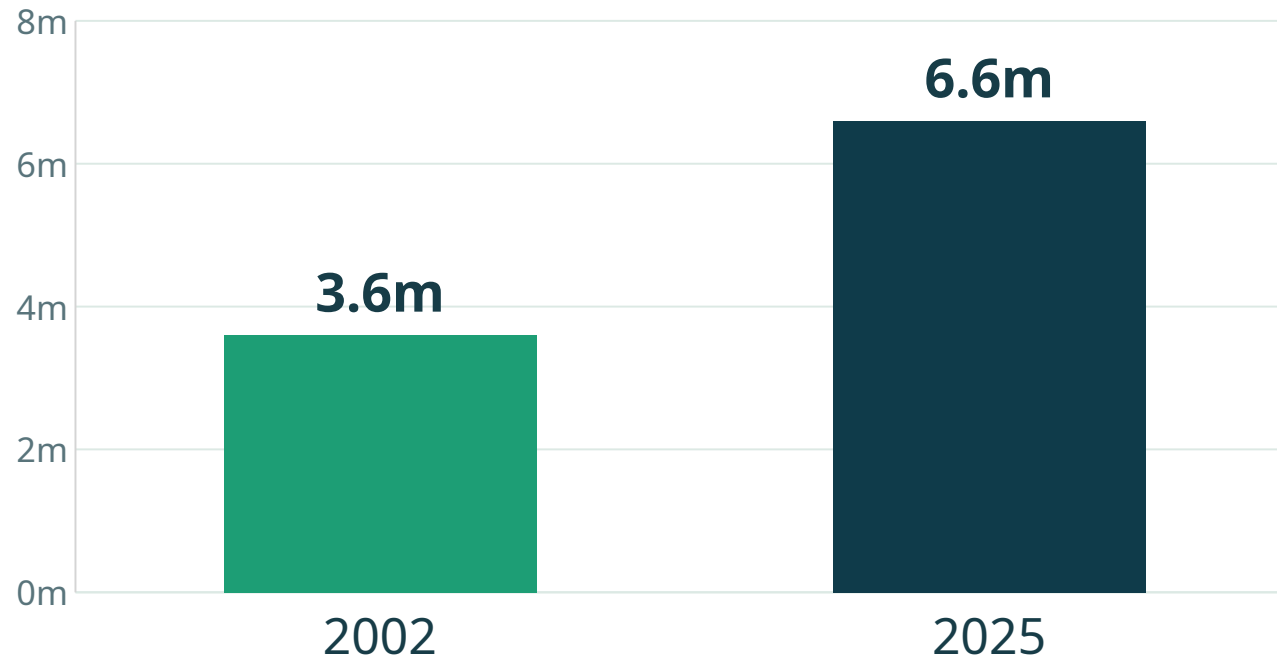
reported medical-aid coverage nationally

A large access gap. Local demand and pricing need testing.

Source: [Stats SA](#), General Household Survey 2025, Figure 5.3, p21. Coverage is not a measure of MyThuso’s paying market.

South Africa's older population is growing

People aged 60 and older, millions



6.6 million

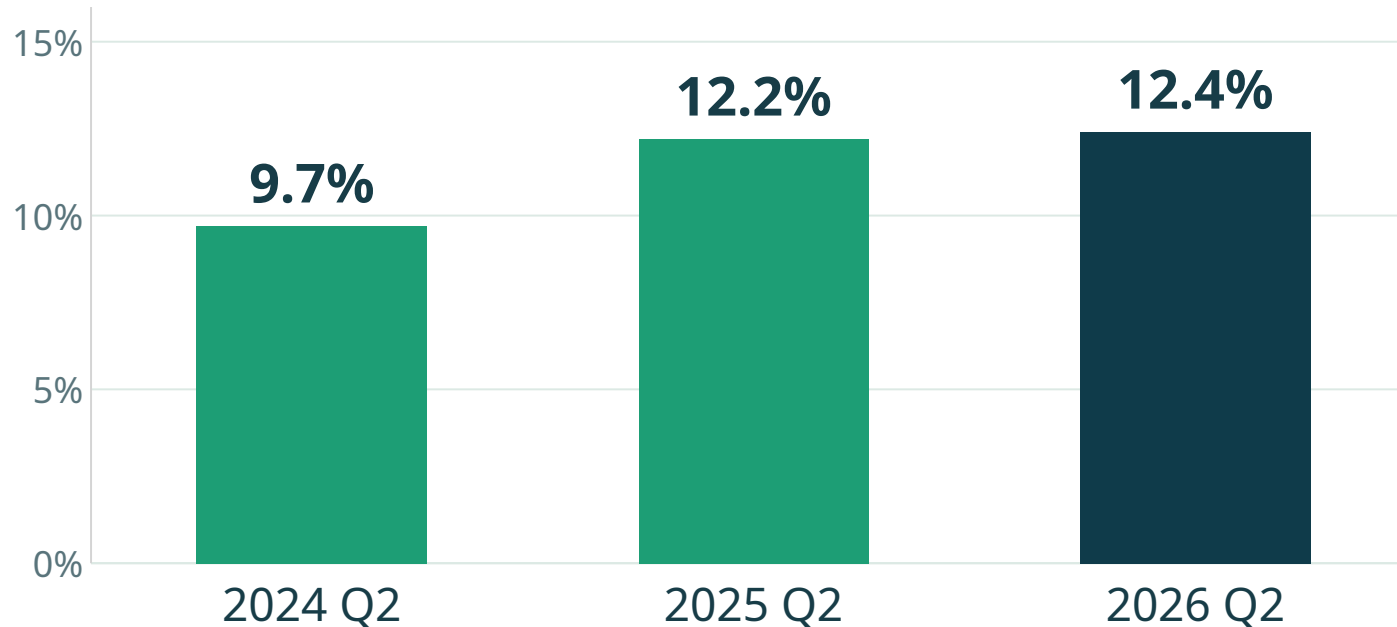
older South Africans
in the 2025 estimate

A focused starting point:
recurring care for a parent,
organised by the family.

Source: [Stats SA](#), Healthy Ageing in South Africa, published 2026. Same-source estimates through 2025, rounded.

Graduate unemployment remains elevated

Official unemployment rate among graduates, all fields, second quarter



12.4%

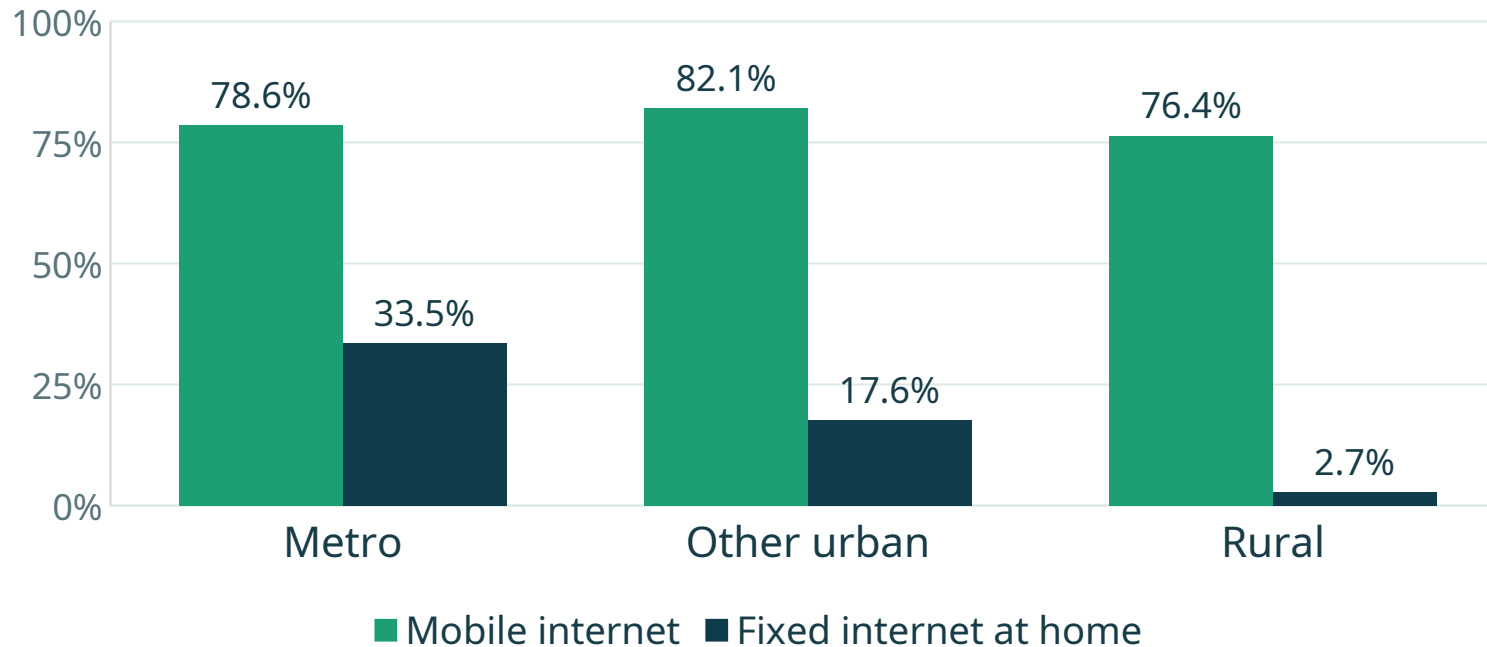
among graduates in
the labour force,
aged 15–64

All fields and career stages.
This is not a doctor or
nurse unemployment rate.

Source: Stats SA QLFS Q2 2025, Figure 8b, p8; Q2 2026, Figure 4b, p6. Same-quarter comparisons.

Mobile-first access, with assisted alternatives

Household internet access by location, 2025. Channels overlap and must not be added.



76.4%

of rural households
reported mobile
internet access

Assisted booking and
low-data flows remain
important.

Source: [Stats SA](#) GHS 2025, Table 14.3, p56. Access does not establish an individual smartphone, affordability or continuous connectivity.

Family care remains the recurring opportunity.

Existing concept prices are preserved; benefits must be costed before any plan is sold.

Essential · R399/mo

One planned nurse visit each month.

Medicine collection and authorised family updates remain proposed benefits.

Recommended first subscription test after reliable per-visit delivery.

Plus · R749/mo

Fortnightly visit concept.

Doctor review, screening and Band-related benefits create additional costs.

Defer release until the full annual benefit liability is priced.

Premium · R1,299/mo

Weekly visit concept.

Home station, carer hours and priority assistance require partners and explicit allowances.

No unlimited urgent care or device availability is assumed.

Subscription benefits can consume the residual.

Revised cash illustration; 26/12 fortnightly and 52/12 weekly visits per month.

Monthly plan	Essential	Plus	Premium
Catalogued concept price	R399	R749	R1,299
Visit frequency / month	1.00	2.17	4.33
Nursing at R224 / visit	R224	R485.33	R970.67
Card fee: $(3.2\% + R2) \times 1.15$	R16.98	R29.86	R50.10
Residual after nursing and fee	R158.02	R233.80	R278.23
Travel + consumables: R40 / visit	R40	R86.67	R173.33
Review: R45 on every visit	R45	R97.50	R195
Remaining before other benefits	R73.02	R49.63	–R90.10

Excludes medicine delivery, screening, hardware, carer hours, urgent visits, acquisition and fixed overhead. No stacking or extra call-outs costed. These are stress tests, not approved plan margins.

One GilbertOne engine. Three clients.

Separate API engine with an on-device deterministic fallback; clinical release remains gated.

Current role

Navigation, preparation and bounded assistance.

The offline deterministic layer stays on every device. Emergency and refusal boundaries must remain firm.

Clinical release

Triage, real-device readings and handover submission remain gated.

Animated readings show synthetic streams; video remains a placeholder. Medication requests are not reconciled prescriptions.

Later investment

Approved device integrations, validated decision support and custom hardware.

No proprietary device, diagnostic accuracy, staffed alert service or regulatory approval is claimed.

Workforce and rural evidence sources

Dates and definitions matter. Click a source title for the original evidence.

SAMATU, 20 January 2026

1,260 unemployed survey respondents after community service. COSATU archive.

DENOSA via COSATU, May 2023

20,000 unemployed nurses. Historical union estimate, not an official total.

eNCA, 12 April 2025

DENOSA interview revisits the 20,000 figure. It does not establish a new census.

Stats SA QLFS, Q2 2025 / Q2 2026

Graduate unemployment, all fields. Denominator: graduate labour force, ages 15–64.

Stats SA GHS 2025

Public-care first contact, medical aid, vehicle ownership and household internet.

MyThuso website and status

Official logo, published launch scope and development-stage service status.

No current official national nurse unemployment total was verified. Unemployment, vacancies, registrations and MyThuso capacity differ.

Evidence and definitions

Sources checked 30 September–2 October 2026. Click a title to open its source.

Stats SA GHS 2025

Medical aid and household internet access

Stats SA Healthy Ageing

Same-source population estimates

Council for Medical Schemes

Open-scheme admission and insurance

Quoro Medical

Published hospital-at-home offer

Medwell

Published home nursing and carer services

Hello Doctor

Published virtual consultation offer

HPCSA telehealth guidance

Booklet 10, December 2021

HPCSA AI guidance

Booklet 20, September 2025

SAHPRA AI/ML communication

Medical-device development and control

Protection of Personal Information Act

Health data, consent and safeguards

South African Nursing Council

Practitioner registration verification

MyThuso website and status

Current published roadmap and launch gates

External facts, original assumptions and proposed recommendations are kept distinct. No traction, outcomes, contracts or approvals are implied.

Cost assumptions investors can inspect.

Cash planning in ZAR over 18 months; no historic spend or sunk software value added to the ask.

Allocation basis	Estimate
Technology: $R75,000 \times 18$ months	R1,350,000
Clinical leads: blended $R66,667/\text{month} \times 18$	R1,200,000 rounded
Operations/support: $R40,000 \times 18$	R720,000
Cloud/SMS/video: blended $R13,333/\text{month} \times 18$	R240,000 rounded
20 standard kits $\times R8,000$ + $R40,000$ calibration/spares	R200,000
3,000 episodes $\times R100$ variable-cost allowance	R300,000
Legal/insurance 350k + growth 300k + admin 140k	R790,000
Base programme / reserve / funding total	R4.80m / R0.70m / R5.50m

Amounts are intended as cash caps including applicable supplier VAT; no VAT recovery assumed. Tax treatment, employment costs, clinician contracting and equipment require professional review and quotes. Reserve is 14.6% of base costs.

Runway funds learning; volume alone is insufficient.

The ask assumes no retained revenue offsets to the programme budget.

R266,667

Average monthly programme use: R4.8m ÷ 18.
R700,000 reserve adds about 2.6 months at that average rate; release timing and procurement change actual runway.

Cash reconciliation: R5.5m funding + R897k patient receipts – R672k nurse payouts – R4.8m programme = R925k before other receipts, timing and tax. Programme includes R300k episode costs; do not count those twice.

Sensitivity at R299: no reviews ≈ R19.70 contribution; 30% reviewed ≈ R6.20; every visit reviewed ≈ –R25.30. Add R15 travel per episode and the 30% scenario becomes ≈ –R8.80. Fixed overhead remains unfunded by these margins.

3,000 visits

At R299: R897,000 gross collections, including R672,000 nurse payouts.
R225,000 remains before payment and other episode costs. Gross collections are not platform revenue.

What changed — and where the numbers come from.

Revision record · existing catalogue values are evidence of planning, not proof of trading.

Funding scope

R9.7m reduced to a proposed R5.5m pilot: R4.2m less (43.3%). Custom hardware deferred; 20 standard kits assumed. Earlier rapid-scale subscriber and visit targets replaced with measurable pilot gates.

Price and fee basis

Service and Mom prices: local catalog contracts. Doctor review planning range R35–R60; model uses R45. Payfast card benchmark: 3.2% + R2 excluding VAT. Assumed 15% VAT added to fee; payout batching/refunds need reconciliation.

Payfast published fees · checked 2 October 2026

xneelo local cloud · supplier benchmark, not a full system quote

BulkSMS pricing · destination/volume quote required

All salaries, travel, consumables, insurance, marketing, infrastructure and reserve allocations are proposed estimates. No provider appointment, contract, compliance certification, diagnostic outcome or traction is asserted. Market research slides retained from the supplied deck; their original dated definitions and citations remain.